

BUNDIBUGYO VIRUS DISEASE OUTBREAK

Democratic Republic of the Congo

Weekly External Situation Report 19, Data as of 20 September 2026

Key Figures at a Glance (Democratic Republic of the Congo)

7 Provinces	6 Provinces	1 902	59.1%	33 520
63 Health Zones Affected	49 Health Zones Active Transmission*	Cases Recovered	Bed Occupancy	Contacts under follow up
821	202	47	1 535	26
Active Cases in Isolation	Health Worker Cases	Health Worker Deaths	Bed Capacity	Testing Laboratories

Note: Health zones with active transmission have reported at least one case in the last 21 days. Active cases in isolation include both suspected and confirmed cases.

Summary

Country	New Confirmed Cases (Last 7 days)	New Confirmed Deaths (Last 7 days)	Confirmed Cases (Total)	Confirmed Deaths (Total)	CFR (%) Confirmed & Probable
Democratic Republic of the Congo	475	222	7 733	3 732	48.3%

Event description

The Bundibugyo virus disease (BVD) outbreak in the Democratic Republic of the Congo has expanded further, with Dungu Health Zone in Haut-Uélé Province, bordering South Sudan, being the latest affected, bringing the total number of health zones affected to 63 since the start of the outbreak. This latest geographic expansion heightens concern about further spread towards international borders, while transmission within the country remains increasingly heterogeneous across affected provinces and health zones. Since [External Situation Report #18](#), a further 475 confirmed cases and 222 confirmed deaths have been reported, bringing the cumulative total to 7733 confirmed cases, including 3732 deaths [crude case fatality ratio (CFR 48.3%)], as of 20 September 2026. Ituri remains the principal focus, although its relative contribution continues to decline, accounting for 76.9% of cumulative confirmed cases, while substantial transmission continues in Nord-Kivu and persists in Haut-Uélé.

At the national level, daily incidence remains high and fluctuating, with the seven-day moving average rebounding in early September before declining in the most recent days. This national pattern masks divergent provincial trajectories.

Ituri continues a gradual decline from its late-July peak but remains at a high level, while Nord-Kivu experienced a marked increase, reaching its highest incidence in mid-September, followed by an apparent decline in recent days. Transmission in Haut-Uélé remains sustained but below its August peak, while Tshopo shows renewed activity from a low baseline. Bas-Uélé continues to report sporadic transmission, with no recent transmission evident in Sud-Kivu. Overall, the epidemic remains geographically heterogeneous, with recent declines in the principal transmission areas occurring alongside persistent low-level transmission and continued geographic expansion to new areas.

Figure 1. Daily national trend in confirmed Bundibugyo virus disease cases, with seven-day moving average, by date of report, Democratic Republic of the Congo, as of 20 September 2026

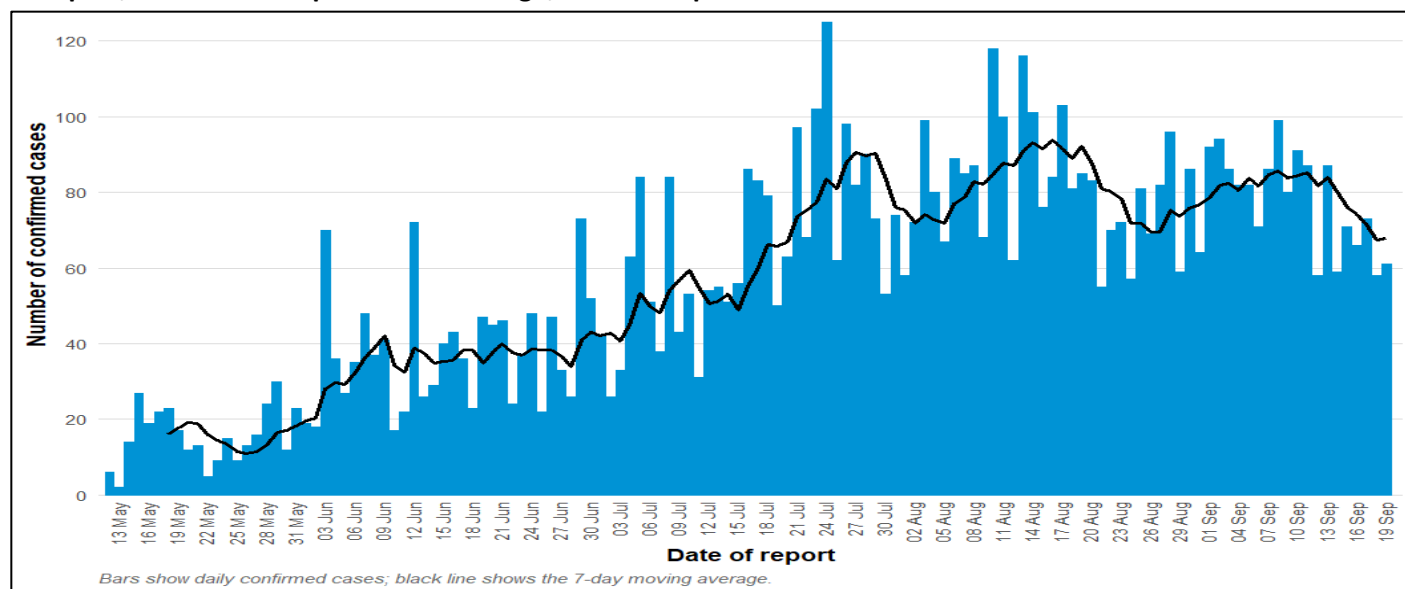
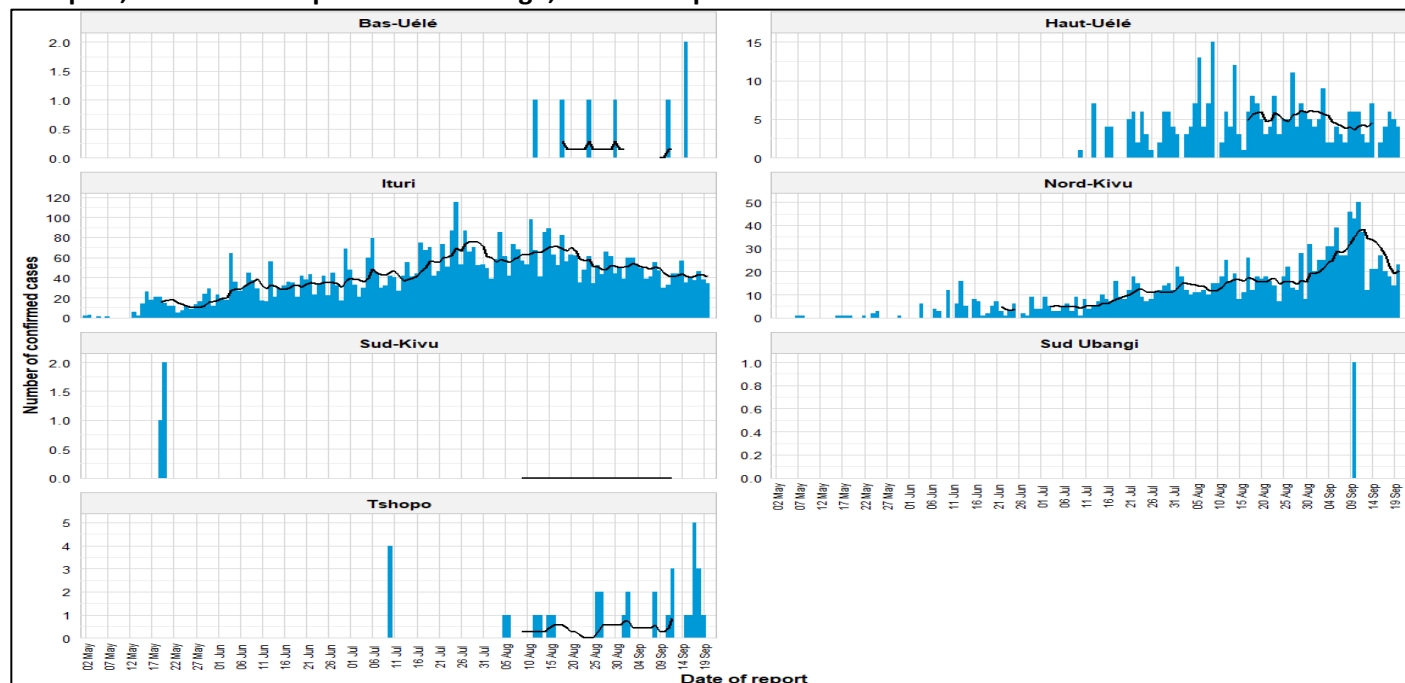


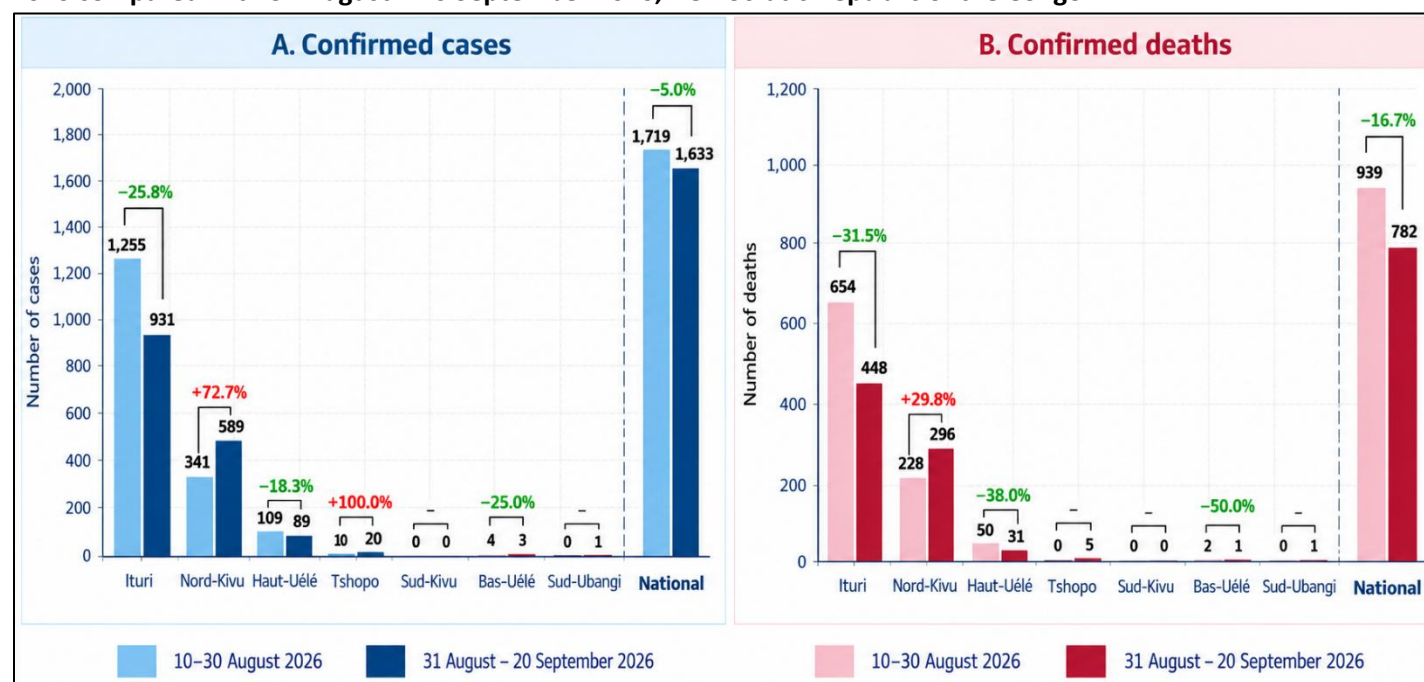
Figure 2. Daily provincial trend in confirmed Bundibugyo virus disease cases, with seven-day moving average, by date of report, Democratic Republic of the Congo, as of 20 September 2026



During the most recent 21 days (31 August – 20 September 2026), a total of 1633 confirmed cases were reported nationally. Compared with 1719 cases during the preceding 21-day period (10 – 30 August 2026), this represents a decrease of 86 cases (–5.0%). Reported cases declined by 25.8% in Ituri, from 1255 to 931, and by 18.3% in Haut-Uélé, from 109 to 89. In contrast, cases increased sharply by 72.7% in Nord-Kivu, from 341 to 589, while Tshopo increased from 10 to 20 cases. Bas-Uélé reported three cases compared with four during the preceding period, while one case was reported in Sud-Ubangi. Consequently, Ituri’s contribution to newly reported cases fell markedly from 73.0% to 57.0%, while Nord-Kivu’s contribution increased from 19.8% to 36.1%; Haut-Uélé’s contribution declined from 6.3% to 5.5%. Overall, the modest 5.0% national decline masks a pronounced geographic redistribution of transmission, with the substantial decline in Ituri increasingly offset by continued high transmission in Nord-Kivu and geographic expansion to new areas.

During the same period, 782 confirmed deaths were reported nationally, compared with 939 deaths during the preceding 21 days, representing a decrease of 157 deaths (–16.7%). The national decline was driven largely by Ituri, where reported deaths decreased from 654 to 448 (–31.5%), while deaths also declined in Haut-Uélé, from 50 to 31 (–38.0%). In contrast, deaths increased substantially in Nord-Kivu, from 228 to 296 (+29.8%). Consequently, Ituri’s contribution to newly reported deaths fell from 69.6% to 57.3%, while Nord-Kivu’s contribution increased from 24.3% to 37.9%; Haut-Uélé’s contribution declined from 5.3% to 4.0%. Tshopo reported five deaths, Bas-Uélé one, and Sud-Ubangi one during the latest period. Overall, the decline in national mortality was driven predominantly by the substantial reduction in Ituri and masks the continued concentration of mortality in Nord-Kivu, which now accounts for more than one-third of newly reported cases and deaths nationally.

Figure 3. Bundibugyo virus disease cases and deaths by province across consecutive 21-day periods, 10 – 30 August 2026 compared with 31 August – 20 September 2026, Democratic Republic of the Congo

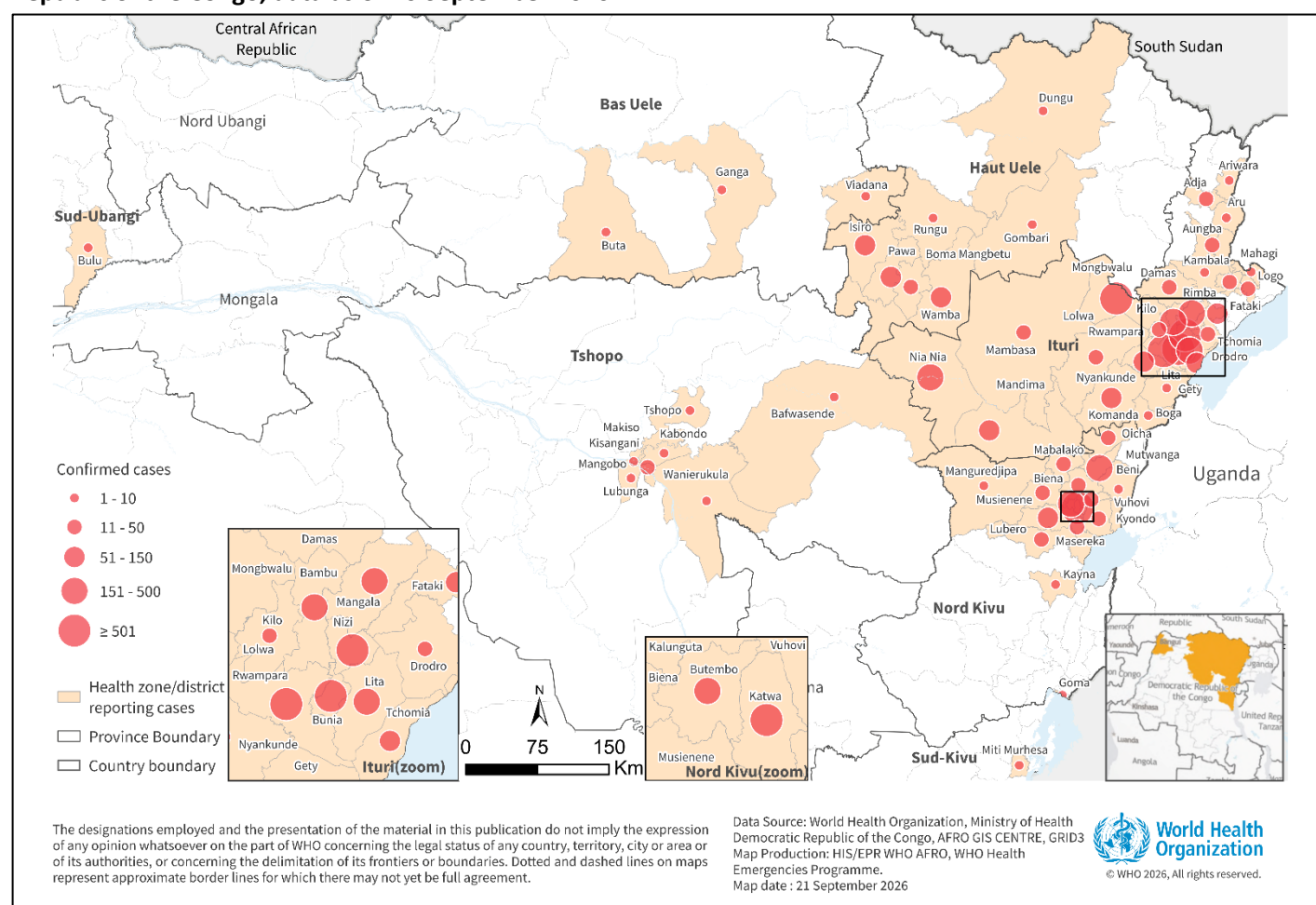


Additionally, during the most recent 21 days (31 August–20 September 2026), 49 of the 63 affected health zones (77.8%) reported at least one new confirmed case, while 14 (22.2%) reported no new cases. Health zones reporting no cases were Adja, Aru, Ariwara, Aungba, Boga, Kambala and Mahagi in Ituri; Goma in Nord-Kivu; Gombari in Haut-Uélé; Lubunga, Tshopo and Wanie-Rukula in Tshopo; Miti-Murhesa in Sud-Kivu; and Buta in Bas-Uélé.

Provincial trends continue to mask important differences between health zones. In Ituri, where cases declined overall between the two consecutive 21-day periods, several health zones showed increasing or sustained transmission. Cases nearly doubled in Komanda, from 36 to 69 (+91.7%), and increased substantially in Mongbwalu, from 44 to 79 (+79.5%), while remaining broadly stable in Lita, from 55 to 57 (+3.6%). In contrast, transmission declined substantially in several major hotspots, including Bunia, from 376 to 265 (–29.5%), Nizi, from 183 to 94 (–48.6%), Rwampara, from 190 to 104 (–45.3%), and Nia-Nia, from 77 to 54 (–29.9%). Mangala also declined from 130 to 91 cases (–30.0%).

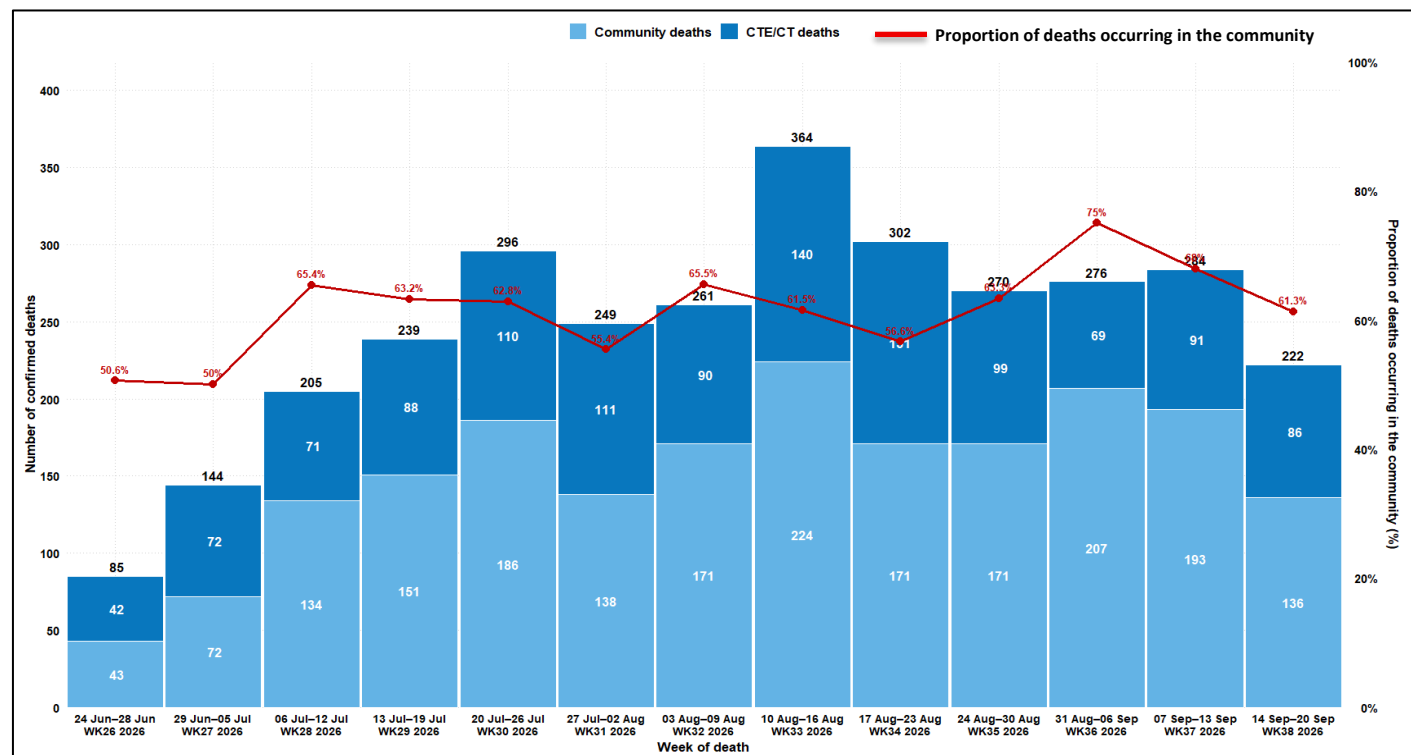
Nord-Kivu showed the opposite pattern, with continued broad-based intensification, although trends varied between individual health zones. Cases more than doubled in Beni, from 62 to 143 (+130.6%), and increased in Butembo, from 64 to 87 (+35.9%) and Musienene, from 31 to 42 (+35.5%). In contrast, Katwa remained at a very high level but declined slightly, from 161 to 154 cases (–4.3%). Haut-Uélé declined overall, but this similarly concealed divergent health-zone trajectories: cases in Pawa more than doubled from 16 to 35 (+118.8%), while declining in Isiro from 50 to 18 (–64.0%) and Wamba from 35 to 25 (–28.6%).

Figure 4. Geographical distribution of cumulative confirmed cases of Bundibugyo virus disease in the Democratic Republic of the Congo, data as of 20 September 2026



Weekly confirmed deaths peaked at 364 during 10 – 16 August, before declining to 302 and 270 over the following two weeks. Deaths subsequently increased to 276 during 31 August – 6 September and 284 during 7 – 13 September, before declining to 222 during 14 – 20 September (–21.8%). This latest decline occurred in both settings, with community deaths decreasing from 193 to 136 (–29.5%) and deaths in treatment facilities declining from 91 to 86 (–5.5%). Consequently, the proportion of deaths occurring in the community fell from 68.0% to 61.3%, continuing the decline from the peak of 75.0% during 31 August – 6 September. Despite this improvement, nearly two-thirds of confirmed deaths in the latest week continued to occur in the community.

Figure 5. Weekly number of confirmed Bundibugyo virus disease deaths by place of death, Democratic Republic of the Congo, epidemiological week 26 – 38, 2026



The geographic distribution of confirmed BVD community deaths has shifted substantially over time. Ituri remained the dominant contributor throughout most of the outbreak, but its share declined markedly in recent weeks as Nord-Kivu’s contribution increased, reaching near parity during 31 August – 13 September. In the latest week, 14 – 20 September, Ituri’s contribution increased to 58.8%, while Nord-Kivu’s declined to 33.8%, with Haut-Uélé accounting for 4.4% and only small contributions from other affected provinces. Overall, the pattern indicates an important geographic redistribution of community mortality, with Nord-Kivu now contributing a substantially greater share than during the earlier phase of the outbreak, despite the recent decline.

Overall, 2,241 community deaths with age and sex information available were analysed. Children aged <5 years accounted for 554 (24.7%) of these deaths, while 1,157 (51.6%) were male and 1,084 (48.4%) were female. During the most recent 21 days, 482 community deaths with age and sex information available were analysed. Of these, children aged <5 years accounted for 146 (30.3%), representing a greater proportional concentration than in the overall distribution. By sex, 251 (52.1%) were male and 231 (47.9%) were female, broadly consistent with the overall sex distribution. The greater representation of children under five among recent community deaths suggests that this age group may be experiencing increasing vulnerability to death before reaching appropriate care, warranting closer investigation of care-seeking, detection and referral pathways among young children.

Figure 6. Weekly proportion of confirmed Bundibugyo virus disease community deaths (non-treatment centre deaths) by province, Democratic Republic of the Congo, epidemiological week 26 – 38, 2026

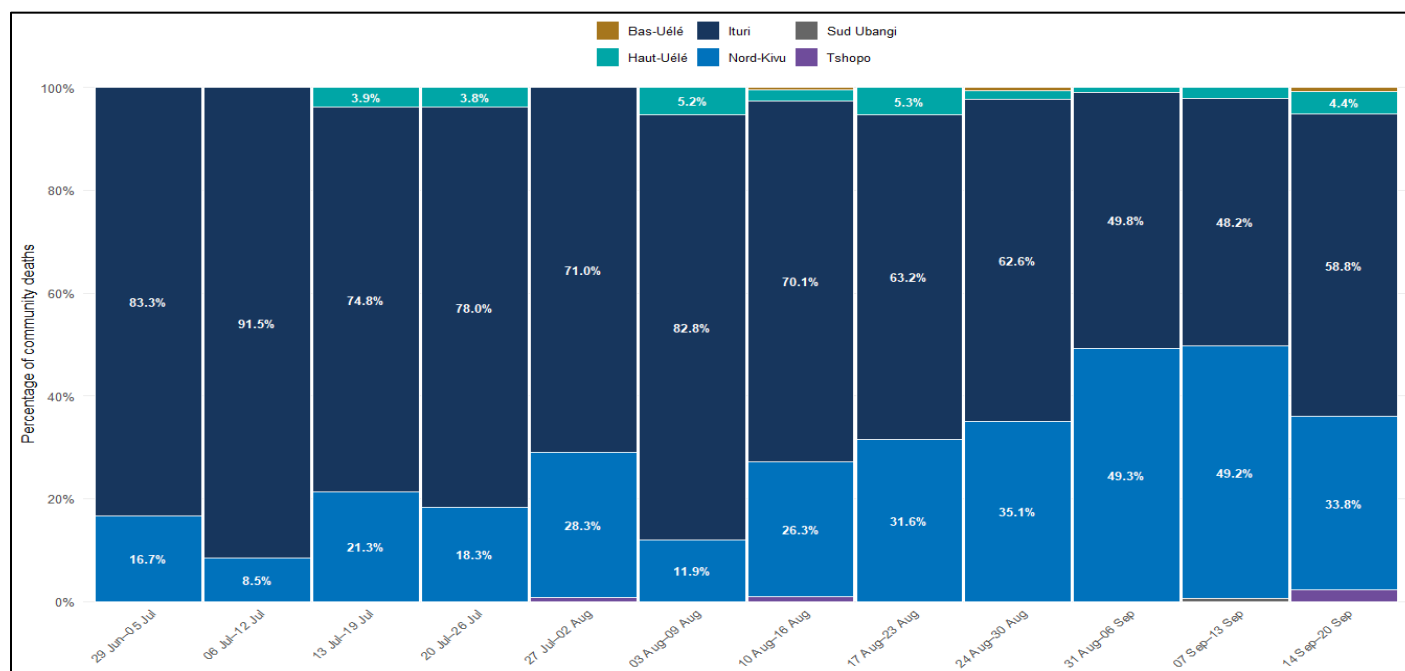
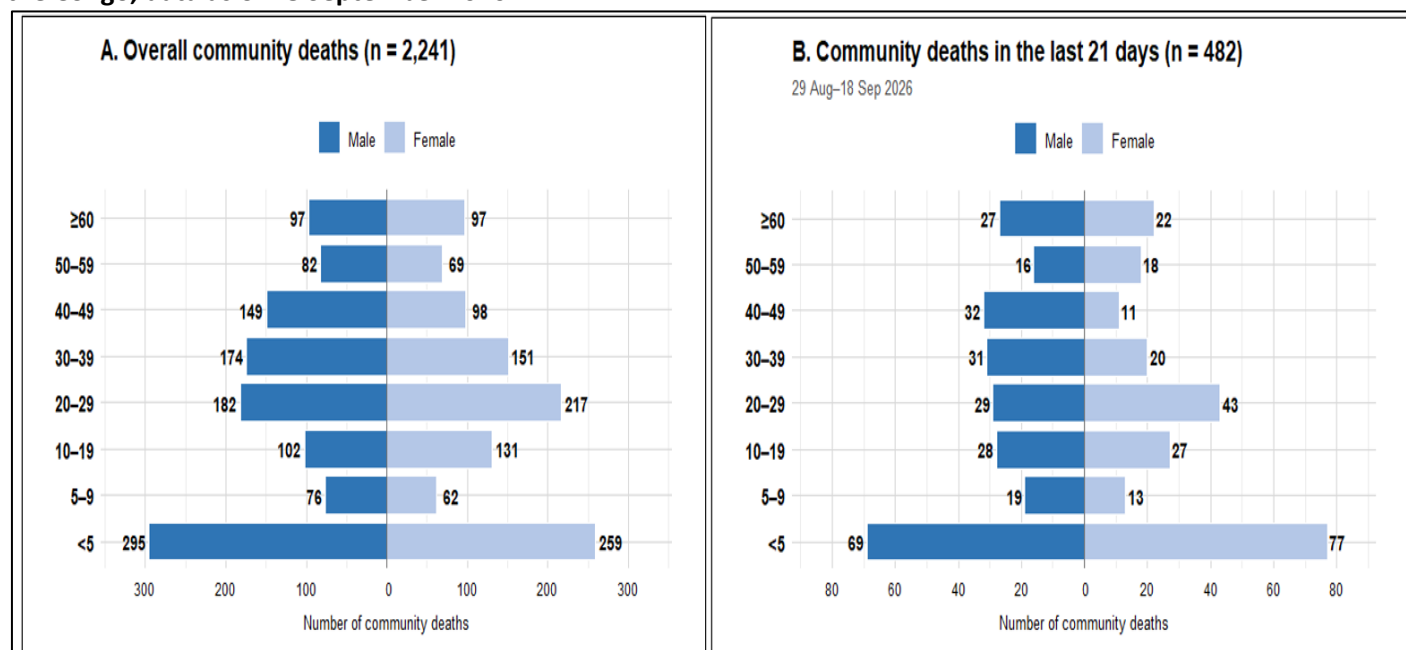


Figure 7. Age and sex distribution of confirmed Bundibugyo virus disease community deaths, Democratic Republic of the Congo, data as of 18 September 2026



Available case investigation information suggests that community deaths may result from multiple barriers along the pathway to appropriate care, including missed diagnosis at peripheral health facilities, self-medication, care-seeking from traditional healers or religious places, refusal of referral, and no prior healthcare contact. Although their relative contribution cannot yet be quantified, these pathways highlight the need to strengthen early case identification, community detection, and timely referral to appropriate treatment.

Risk Assessment

The risk of further spread remains very high within the Democratic Republic of the Congo, reflecting sustained transmission, continued geographic expansion, high mortality, population mobility, insecurity and persistent response challenges. A two-week invasion-risk forecast identified 20 previously unaffected health zones at elevated risk of transmission. Dungu, which was among the health zones identified as high risk, has since reported confirmed transmission. The remaining 19 health zones are Rethy, Watsa, Nyarambé, Biringi, Makoro, Karisimbi, Angumu, Rutshuru, Nyiragongo, Niangara, Linga, Kirotshe, Bafwagbogbo, Alimbongo, Jiba, Poko, Rwanguba, Kamango and Kibirizi. This development reinforces the value of risk-based preparedness and readiness measures in health zones identified as being at elevated risk.

The risk is considered high for neighbouring countries sharing land borders with the Democratic Republic of the Congo and low elsewhere in Africa and globally. The second IHR Emergency Committee, convened on 18 August 2026, also reviewed the evolving situation and emphasized that the outbreak remains far from controlled, and continues to constitute a Public Health Emergency of International Concern.

Public health response

Coordination

- Joint WHO–Africa CDC–partner coordination continues through the Continental Incident Management Support Team (CIMST), with increasing emphasis on translating health-zone intelligence into coordinated field action. Current priorities include strengthening the response in North Kivu while maintaining response capacity in Ituri, with attention to staffing, community linkages and other operational gaps.
- The CIMST has planned an intra-action review and revision of the continental response plan, with completion targeted by the end of September to inform implementation beginning October 2026. The process is intended to assess collective response performance, identify gaps and lessons, strengthen accountability and reassess preparedness in high-risk and currently unaffected areas.
- At country level, the Government of the Democratic Republic of the Congo has adopted a revised 180-day multisectoral response plan, providing the operational framework for the next six months. The plan prioritizes surveillance and contact tracing, clinical care, community engagement and risk communication, logistics, and preparedness in at-risk areas, with implementation requirements estimated at US\$1.3 billion.
- A broad network of partners continues to support implementation across affected and at-risk areas, including UNICEF, International Organization for Migration (IOM), Médecins Sans Frontières (MSF), Alliance for International Medical Action (ALIMA), International Medical Corps (IMC), Samaritan's Purse, the European Union, Global Outbreak Alert and Response Network (GOARN), Emergency Medical Teams and StandBy partners, supporting field operations, laboratories, logistics and specialist deployments.

Operations Support and Logistics (OSL)

- WHO continue to expand its operational footprint to support the evolving outbreak response, with 339 experts in active deployment as of 17 September 2026, compared with 234 in July and 92 in June. Deployments are concentrated in the main transmission areas, with operational presence through permanent teams or surge missions in 54 health zones and coverage of all major hotspots. The largest deployment remains in Ituri, while personnel and technical expertise are increasingly being positioned in Nord-Kivu and other emerging

transmission areas.

- The emergency supply pipeline increased to US\$17.53 million, up from US\$12.84 million (+US\$4.69 million; +36.5%). Supplies worth US\$5.79 million (33%) have been delivered, US\$7.63 million (44%) are in transit, and US\$4.11 million (23%) are not yet ready.
- Cumulative delivered cargo increased to 382.3 metric tonnes, up from 355.0 metric tonnes (+27.3 tonnes), and to 2205.9 m³, up from 2014.4 m³ (+191.5 m³). The value of delivered supplies increased by approximately US\$0.44 million, from US\$5.35 million to US\$5.79 million.
- Critical commodities continued to reach the Ituri and Kivu hubs. Deliveries now include 82 274 hooded coveralls, 7.23 million examination gloves, 158 800 Level 3 gowns, 477 hospital beds, 10 076 infrared thermometers, 8640 manual PCR tests and 20016 RadiOne PCR tests. The most notable increases were examination gloves, from 5.53 million to 7.23 million (+1.70 million), and hospital beds, from 261 to 477 (+216). The in-transit pipeline now includes 568 530 coveralls, 3.0 million gloves, 303 000 gowns, 284 hospital beds and 8000 infrared thermometers.
- Personal protective equipment and other infection prevention and control supplies remains the largest procurement category, increasing from US\$5.32 million to US\$6.80 million (+US\$1.48 million). Laboratory supplies increased from US\$2.08 million to US\$2.65 million, while medical equipment remained approximately US\$1.97 million and logistics remained approximately US\$1.37 million.
- Despite continued deliveries, the share of the pipeline delivered or in transit decreased from 92% to 77%. Supplies not yet ready increased from US\$1.09 million (9%) to US\$4.11 million (23%). Procurement remains highly concentrated, with the principal supplier accounting for approximately 83% (US\$11.84 million) of order value.

Surveillance

- Surveillance activities continued across affected and at-risk areas, including active case finding, alert management and investigation, contact tracing, mortality surveillance, community-based surveillance, and surveillance at points of entry (PoEs) and points of control (PoCs).
- From 14 – 20 September, a total of 15 668 alerts were reported, averaging 2238 alerts per day, an increase of 14.3% from 13 709 (1958 per day) during 7 – 13 September. Alert verification improved from 82.9% to 84.8% (13 285 / 15 668). Of 3687 validated suspected cases, 3667 (99.5%) were investigated, up from 95.6% in the preceding week. However, among those investigated, only 1284 (35.0%) were isolated and transferred to treatment centres within 24 hours, declining sharply from 50.4%. Overall, alert reporting and investigation improved, but the widening gap between investigation and timely isolation and referral remains a critical weakness in the alert-management cascade.
- From 14 – 20 September, contact-tracing workload increased substantially, with an average of 32 292 contacts requiring daily follow-up, compared with 26 373 during 7 – 13 September, an increase of 22.4%. Despite the increased workload, average follow-up performance remained broadly stable, improving slightly from 85.6% to 85.8%, but remained below the ≥95% target. As of 20 September, a total of 33 520 contacts were under follow-up, of whom 27 281 (81.4%) were seen. Performance varied across provinces, ranging from 100% in Sud-Ubangi and 91.5% in Bas-Uélé to 83.9% in Tshopo, 83.3% in Nord-Kivu and 79.9% in Ituri. The ability to maintain overall follow-up performance despite a substantial increase in workload is encouraging, although persistent coverage

gaps, particularly in the major transmission areas of Ituri and Nord-Kivu, remained.

- At points of entry (PoEs) and points of control (PoCs), 257,106 travellers were recorded on 20 September, with 97.5% screened. Three alerts were detected, two in Ituri and one in Nord-Kivu, all of which were validated as suspected cases and investigated within 24 hours; two (66.7%) were transferred to treatment centres, while one suspect refused transfer. Overall reporting completeness reached 95.2%, with full reporting in Nord-Kivu and Tshopo, 93.3% in Ituri and 85.7% in Haut-Uélé. All strategic PoEs/PoCs were reported as functional.

Laboratory

- Diagnostic capacity continued to expand and decentralize, increasing from two laboratories at the onset of the outbreak to 26 active laboratories. Of these, ten primarily use open RT-PCR platforms and 16 primarily use RadiOne near-point-of-care PCR platforms, providing a reported combined testing capacity of more than 4700 samples per day. The set up of additional laboratories and diagnostic platforms is planned.
- Laboratory validation of candidate BVD diagnostic tests under the BRIDGE-Access initiative commenced at the Institut National de Recherche Biomédicale (INRB) in Kinshasa on 14 September, supporting the evaluation of BDBV-specific and broader filovirus assays, including near-point-of-care tests.. Results will contribute to generating the clinical evidence needed to inform deployment, procurement, and regulatory pathways for outbreak diagnostics.
- Priorities are now to consolidate and sustain the expanded network through implementation of the updated decentralization plan; reliable supplies, equipment, training and supervision; and improved turnaround-time data capture, specimen management and partner reporting. WHO is procuring approximately US\$1.5 million in laboratory supplies and critical equipment to support new and existing laboratories.

Case Management

- Case-management capacity includes 49 treatment centres/units providing 1535 active beds, with 821 patients hospitalized or in isolation as of 20 September, corresponding to a national bed occupancy of 59.1%.
- Case management capacity remains uneven across affected provinces. Ituri reported 442 hospitalized patients with 43.5% bed occupancy, while Nord-Kivu remains under the greatest pressure, with 316 patients hospitalized and 91.2% occupancy in standardized treatment structures. Haut-Uélé and Tshopo reported lower occupancy at 32.6% and 38.7%, respectively, while six patients were hospitalized in health facilities in Bas-Uélé.
- Important capacity gaps persist, including saturation of suspect-care capacity in several Ituri health zones, 34.2% of hospitalized patients in Nord-Kivu being managed outside standardized treatment centres and no available beds in Butembo and Katwa, limited technical capacity and lack of safe blood in Tshopo, and the absence of a treatment centre and ambulance in Bas-Uélé.
- Cumulative recoveries increased to 1902 as of 20 September 2026, up from 1726 on 13 September, representing 176 additional recoveries over the reporting period.
- Multi-sectoral, multi-partner training that was launched two weeks ago continues to increase the capacity of trained staff in dedicated treatment centres. Two cohorts of 30 individuals completed (total of 60 trained), and two more cohorts to be trained this week.
- The survivor programme opened in Bunia on 19 September to monitor and address the specific medical needs of recovered patients, with additional sites planned to expand access to survivor care.

Research and Development

- Following the 17 August 2026 allocation of 70 000 doses of Ervebo® to the Democratic Republic of the Congo, 20 000 doses were allocated to the Phase 3 clinical trial, 2,000 to the BRAVO observational vaccine-effectiveness study among healthcare workers in Ituri, and 30 000 for healthcare and frontline workers under separate research protocols with informed consent.
- Ervebo® vaccination of healthcare and frontline workers continues to expand geographically with 12 active health zones in 5 provinces. In total 4,944 people have been vaccinated. Ultra-cold-chain equipment has also been received in Buta.
- The BRAVO observational study was launched in Bunia on 20 September to support ongoing research activities related to the outbreak.
- Ervebo® use for BVD remains within a research context. Following its 19 August review, the WHO Strategic Advisory Group of Experts on Immunization (SAGE) concluded that available evidence was insufficient to support programmatic use for BVD and that efficacy against BVD in humans remains unknown. Research on Ervebo® effectiveness against BVD is ongoing, alongside evaluation of BDBV-specific vaccine candidates including Oxford's ChAdOx1 and Moderna's mRNA candidates.
- Therapeutic and prophylaxis research continues to expand through the PARTNERS treatment trial and the EBO-PEP post-exposure prophylaxis study. As of 20 September 2026, the PARTNERS trial has enrolled 515 participants across five treatment centres. Preliminary results are expected in approximately 4 – 6 weeks, once most participants have completed 28-day follow-up. The EBO-PEP study has enrolled 299 participants across the Rwampara and CME sites, including 226 in the obeldesivir/placebo arm and 73 in the remdesivir arm.
- The MBOTE clinical characterization study continues recruitment across four centres to characterize the clinical presentation, progression and outcomes of BVD and complement evidence generated through ongoing vaccine, therapeutic and prophylaxis studies.

Infection Prevention and Control (IPC) :

- IPC performance across healthcare facilities remains a major concern. During the week, 509 IPC assessments were conducted across 202 healthcare facilities (HCFs), with a mean score of only 39.5%. Of these, 144 HCFs scored below 50%, 51 scored between 50% and 80%, and only seven met the 80% threshold. Key gaps were identified in the availability of personal protective equipment (PPE), isolation capacity and training of healthcare workers, while patient placement, injection safety, waste treatment, intrahospital surveillance and reprocessing of reusable medical materials and equipment were the only components scoring above 50%.
- Six healthcare workers were infected during the week, and all underwent in-depth investigations to assess potential exposures and circumstances of infection.
- Safe and dignified burial (SDB) coverage remained relatively high overall, with 1176 SDBs conducted following 1359 death alerts, representing 86.5% coverage. However, 176 death alerts did not result in an SDB and eight burials were conducted more than 24 hours after notification, with important localized gaps persisting. Fataki and Mandima health zones in Ituri recorded failure rates of 53.7% and 4.3%, respectively.
- Insecurity continued to constrain SDB activities in Mandima, while six community SDB teams were trained and

deployed in Fataki to address the coverage gap.

- Environmental IPC activities continued at scale, although coverage remained incomplete. A total of 763 of 942 targeted households (81.0%) and 309 of 352 targeted HCFs (87.8%) were disinfected. Response teams distributed 91 IPC kits to HCFs and 268 hygiene kits, including 229 to households and 39 to public places, and established 14 screening stations in HCFs.
- Family resistance, insufficient transport for response teams and difficulties locating households remained the main reported barriers to household disinfection.

Risk Communication and Community Engagement (RCCE)

- Community engagement activities remained extensive across the main affected provinces, using household visits, educational discussions, mass sensitization, community dialogues, media activities and engagement with local, religious and other community leaders. In Ituri alone, activities on 20 September included 56 433 household visits reaching 95 073 people, alongside educational and mass-sensitization activities.
- Efforts increasingly focused on building trust and addressing resistance, including direct engagement with community and religious leaders and visits to treatment centres. In Ituri, 47 local leaders visited the Nia-Nia Ebola treatment centre, while community leaders in Nord-Kivu were mobilized to promote adherence to response measures.
- Important community-engagement gaps persist, including incomplete reporting, unresolved feedback and incidents, refusal of case and contact listing, resistance to decontamination and denial of the disease in some areas. In Ituri, reporting completeness was only 35.7% (10/28 health zones) on 20 September, while 79.8% of feedback and 66.7% of reported community incidents were resolved. These gaps continue to affect acceptance and timely implementation of response interventions.

Preventing and Responding to Sexual Exploitation, Abuse, and Harassment (PRSEAH)

- PRSEAH activities continued during the week, with briefings and sensitization targeting response personnel, community-based workers and communities, alongside dissemination of information on expected conduct and available reporting mechanisms. In Ituri, activities included response personnel across Bunia, Nizi, Rwampara and Mongbwalu, while community actors were also engaged in Bas-Uélé.
- Community sensitization was strengthened, including engagement of women and other potentially vulnerable groups. On 14 September alone, 579 community members were reached in Ituri, including 409 women, while response personnel received PRSEAH briefings and sensitization.
- Awareness of reporting mechanisms remains limited. Only 13 of 36 health zones (36.1%) in Ituri were reported to be familiar with the mechanism for reporting sexual exploitation and abuse, indicating substantial gaps in access to and awareness of safeguarding channels.
- Efforts to strengthen PRSEAH coverage and coordination continued, including engagement of response partners and expansion of awareness activities. However, further strengthening is needed to complete partner mapping and alignment, expand the availability of sensitization materials, and improve access to essential information technology and operational resources to ensure consistent integration of PRSEAH safeguards across the

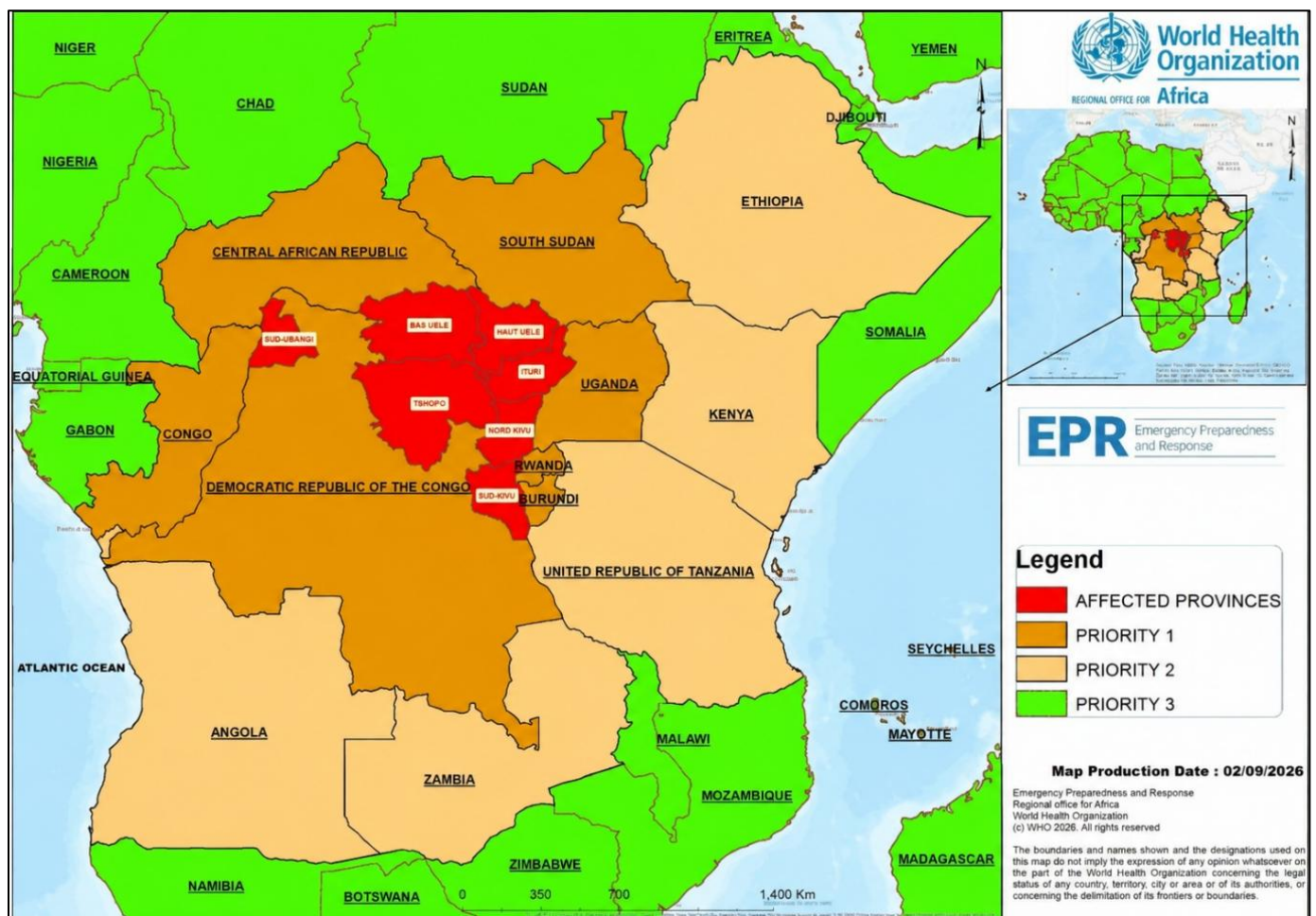
response

Public health preparedness

Operational Readiness

- Based on regional cross-border risk modelling and the evolving epidemiological situation, countries have been recategorized by priority for response and preparedness. The Democratic Republic of the Congo is classified as the response country (Priority 1A); Uganda, Central African Republic, South Sudan, Republic of the Congo, Burundi and Rwanda as Priority 1B countries; and Angola, Ethiopia, United Republic of Tanzania, Kenya, and Zambia as Priority 2 countries.

Figure 6. Regional prioritization of countries for Bundibugyo virus disease preparedness and response, as of 13 September 2026



- Ten out of 11 high-priority countries completed repeat readiness assessments, reflecting continued efforts to strengthen national preparedness capacities: Angola, Burundi, Central African Republic, South Sudan, Tanzania, Kenya, Republic of the Congo, Rwanda, Uganda and Zambia, average scores have increased from 55% to 61%.
- Regional technical deployments continued to support preparedness and response capacity in affected and high-risk Member States, including Central African Republic, Democratic Republic of the Congo and Uganda.

Border Health, Travel and Mass Gatherings

- Screening and public health measures have been reinforced at airports, high-risk border crossings, major ports and internal transit routes in the Democratic Republic of the Congo and Uganda. National authorities, IOM, and partners continue to support the implementation of WHO border health guidance, strengthen cross-border coordination, and facilitate the timely detection, reporting and referral of suspected cases at PoEs and PoCs along priority travel corridors.
- Cross-border preparedness and response are being strengthened through enhanced coordination among the Democratic Republic of the Congo, Uganda, the Central African Republic and South Sudan to improve the early detection of cases and reduce the risk of cross-border spread. Priority actions include strengthening coordinated cross-border disease surveillance, event-based surveillance, active case finding, joint investigations and contact tracing where required, and real-time information sharing through regular cross-border coordination meetings and harmonized reporting mechanisms.
- Priority operational actions include rapid assessments and mapping of PoEs, strengthening surveillance along high-risk cross-border mobility corridors, including informal crossings, and major congregation points; and ensuring readiness capacities through the provision of trained personnel, equipment, PPE, referral mechanisms, disinfectants, risk communication materials and contingency plans for the safe management of suspected cases.
- The postponement of mass gatherings is advised in areas with documented BVD transmission until transmission is interrupted. Event-specific risk assessments should be conducted using the WHO all-hazards mass gathering risk assessment tool. These assessments should be complemented by implementation of measures before, during, and after events, as outlined in WHO guidance for mass gatherings attended by individuals from areas with documented Bundibugyo virus detection, including the establishment of referral pathways and RT-PCR testing capacity.
- Event organisers should distribute participant information to all attendees before and during the event, covering BVD symptoms, what to do if feeling unwell, and available health services, in line with [WHO guidance for mass gathering events attended by individuals from areas with documented BDBV detection](#).

Situation interpretation

The outbreak remains uncontrolled and increasingly geographically dispersed, expanding the operational footprint of the response and placing additional pressure on already stretched resources. The emergence of transmission in Dungu, a health zone bordering South Sudan that was previously identified as being at high risk, further heightens the potential for cross-border spread and reinforces the need to accelerate readiness in areas where further spread is anticipated. At the same time, community mortality remains a major concern, with nearly two-thirds of recent deaths occurring in the

community, indicating persistent delays in detection, referral and access to appropriate care. Children under five are disproportionately represented among community deaths, suggesting particular vulnerabilities in early recognition of illness and timely care-seeking and referral for young children. The multiple pathways leading to community deaths, including missed recognition at health facilities, self-medication, alternative care-seeking, refusal of referral and no prior healthcare contact, indicate that reducing mortality will require interventions across the entire pathway to care. Priorities should include stronger community detection, early recognition at peripheral health facilities, rapid referral and improved community trust and acceptance, alongside targeted investigation and interventions to address the specific barriers contributing to community deaths among children under five.

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